DRUGS · HEAD AND NECK ONCOLOGY

Docetaxel and paclitaxel

Docetaxel and paclitaxel stabilise microtubules and stop cell division. Docetaxel is the "T" in TPF induction chemotherapy (docetaxel, cisplatin, 5-FU), used in larynx preservation protocols and to shrink advanced tumours before definitive treatment; paclitaxel is used weekly in palliation and with radiotherapy in some regimens. Both need steroid premedication and cause neutropenia and neuropathy.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

DOCETAXEL AND PACLITAXEL

	SUMMARY
Class	Taxanes
How they work	Bind beta-tubulin and stabilise microtubules, arresting cells in mitosis
ENT uses	TPF induction chemotherapy for larynx and hypopharynx preservation and for selected locally advanced tumours; palliative treatment of recurrent and metastatic disease (alone, with platinum, or after immunotherapy); some sinonasal and nasopharyngeal regimens
Dose	TPF: docetaxel 75 mg/m2 and cisplatin 75 mg/m2 on day 1, 5-FU 750 mg/m2 daily on days 1 to 5, every 3 weeks for 2 to 3 cycles; paclitaxel 80 mg/m2 weekly in palliation
Premedication	Dexamethasone (docetaxel: fluid retention and hypersensitivity); antihistamines and steroids (paclitaxel: hypersensitivity to the Cremophor vehicle)

2 CHAPTER TWO Using them in ENT

Practice.

2.1 Larynx preservation

In advanced laryngeal and hypopharyngeal cancer, responders to TPF induction proceed to radiotherapy or chemoradiotherapy with preservation of the larynx; non-responders proceed to laryngectomy. Patients with a non-functioning larynx (aspiration, airway obstruction needing tracheostomy, cartilage destruction) are usually better served by laryngectomy.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

Neutropenia and febrile neutropenia (granulocyte colony-stimulating factor support with TPF), peripheral neuropathy, hypersensitivity reactions, fluid retention (docetaxel), alopecia, nail changes, mucositis.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- The TPF regimen and its role in **larynx preservation**.

- Premedication and neutropenia.

4.2 Common pitfalls

COMMON PITFALLS

- Offering larynx preservation to a patient with a non-functioning larynx.

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